

Pelvic Pain During Early Pregnancy

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Early in pregnancy, many women have pelvic pain. Pelvic pain refers to pain in the lowest part of the abdomen (pelvis). Pain in the middle or upper part of the abdomen, in the area of the stomach and intestine, is called abdominal pain. However, sometimes women have trouble discerning whether pain is mainly in the abdomen or pelvis. Causes of abdominal pain during pregnancy may or may not be related to the pregnancy.

Pelvic pain in pregnancy may be related to normal changes of pregnancy or may be a sign of a problem. It can occur normally as the bones and ligaments shift and stretch to accommodate the fetus. Normal pain may be sharp or crampy (like menstrual cramps) and may come and go. Usually, pelvic pain that is not severe or persistent is not a cause for concern.

Pain that is caused by a pregnancy complication may be accompanied by other symptoms, including [vaginal bleeding](#). Women should contact their doctor if they have any bleeding during pregnancy. In some disorders, such bleeding can be severe, sometimes leading to dangerously low blood pressure ([shock](#)). Pain that is severe, persistent, or in the middle or upper abdomen may be caused by a pregnancy complication or another disorder and is also a reason to contact a doctor.

Causes of Pelvic Pain in Early Pregnancy

During pregnancy, pelvic pain may result from disorders that are related to

- The pregnancy
- The female reproductive system but not the pregnancy
- Other organs, particularly the digestive tract and urinary tract

Sometimes no particular cause is identified.

The **most common pregnancy-related causes** of pelvic pain during early pregnancy are

- Normal changes of pregnancy
- A [miscarriage](#) (spontaneous abortion)
- An [ectopic pregnancy](#) (a pregnancy that is implanted outside the uterus, for example, in a fallopian tube)— the most serious cause of pelvic pain during early pregnancy

An ectopic pregnancy is life threatening. A pregnancy implanted outside the uterus will not develop normally. As the pregnancy grows in a fallopian tube or other body part (such as the ovary, cervix, or abdomen), it may cause that part to burst. This leads to severe pain and heavy bleeding, which can result in severe harm or even death to the pregnant woman. When an ectopic pregnancy is diagnosed, the treatment is to end the pregnancy with medications or remove it with surgery.

Pelvic pain may also occur when an ovary twists around the ligaments and the tissues that support it, cutting off the ovary's blood supply. This disorder, called [adnexal torsion](#), is not related to the pregnancy but is more common during pregnancy. During pregnancy, the ovaries enlarge, making an ovary more likely to twist.

Common causes of pelvic pain in general, which are also common causes during pregnancy, are digestive and urinary tract disorders, including the following:

- [Urinary tract infections](#) (UTIs)
- [Kidney stones](#)
- [Gastroenteritis](#) (infection of the digestive tract)
- [Irritable bowel syndrome](#)
- [Appendicitis](#)
- [Inflammatory bowel disease](#)

Urinary tract infections occur more frequently during pregnancy, and there is an increased risk of the infection spreading from the bladder to a kidney and becoming a more serious infection.

Risk factors

Various characteristics (risk factors) increase the risk of some pregnancy-related disorders that cause pelvic pain.

For **miscarriage**, risk factors include the following:

- Age over 35
- One or more [miscarriages](#) in previous pregnancies
- [Cigarette smoking](#)
- Use of illicit drugs, such as [cocaine](#), or possibly other substances, such as [alcohol](#)
- Abnormalities in the uterus, such as [fibroids](#), scarring, or an abnormal uterine shape

For **ectopic pregnancy**, risk factors include the following:

- A previous ectopic pregnancy (the most important risk factor)
- Previous abdominal surgery, especially surgery for permanent sterilization (tubal ligation)
- Fallopian tube abnormalities (for example, a swollen fallopian tube, called a hydrosalpinx)
- Current use of an [intrauterine device](#) (IUD)
- Use of [assisted reproductive techniques](#) (in vitro fertilization) to conceive current pregnancy

Additional risk factors for ectopic pregnancy include history of [sexually transmitted infection](#) or [pelvic inflammatory disease](#), current use of estrogen/progestin oral contraceptives, cigarette smoking, infertility, and prior [miscarriage](#) or intentional termination of pregnancy (induced [abortion](#)).

Evaluation of Pelvic Pain in Early Pregnancy

If a pregnant woman has pelvic or abdominal pain, a doctor will evaluate whether this is caused by normal changes of pregnancy or if there is a problem. If there is sudden, very severe pain in the lower abdomen or pelvis, doctors must quickly try to determine whether prompt surgery is required—as is the case when the cause is a ruptured ectopic pregnancy or appendicitis.

Warning signs

In pregnant women with pelvic pain, the following symptoms are cause for concern:

- [Vaginal bleeding](#)

- Fever and chills, particularly if accompanied by pain with urination, upper back (flank) pain, or a vaginal discharge that contains pus
- Pain that is severe and is made worse with movement
- Fainting, light-headedness, or a rapid heart rate—symptoms that suggest very low blood pressure

When to see a doctor

Women with warning signs should see a doctor immediately.

Women without warning signs should try to see a doctor within a day or so if they have pain or burning during urination or pain that interferes with daily activities. Women with only mild discomfort and no other symptoms should call the doctor. The doctor can help them decide whether and how quickly they need to be seen.

What the doctor does

To determine whether emergency surgery is needed, doctors first check the woman's blood pressure and temperature and ask about key symptoms, such as vaginal bleeding. Doctors then ask about other symptoms and the medical history. They also do a physical examination. What they find during the history and physical examination often suggests a cause and the tests that may need to be done (see table

Some Causes and Features of Pelvic Pain During Early Pregnancy

Doctors ask about the pain:

- Whether it begins suddenly or gradually
- Whether it occurs in a specific spot or is more widespread
- Whether moving or changing positions worsens the pain
- Whether it is crampy and whether it is constant or comes and goes

Doctors also ask about the following:

- Other symptoms, such as vaginal bleeding, vaginal discharge, a need to urinate often or urgently, vomiting, diarrhea, and constipation
- Previous pregnancy-related events (obstetric history), including past pregnancies, miscarriages, and intentional terminations of pregnancy (induced abortions) for medical or other reasons
- Risk factors for miscarriage and ectopic pregnancy

The physical examination focuses on the pelvic examination (examination of the external and internal reproductive organs and sometimes the rectum). Doctors also gently press on the abdomen to see whether pressing causes any pain.



Some Causes and Features of Pelvic Pain During Early Pregnancy

Cause	Common Features*	Diagnostic Approach†
<u>Normal changes of pregnancy</u> , including stretching and growth of the uterus and surrounding connective tissues	A crampy sensation or pressure in the lower abdomen, pelvis, and/or lower back Sometimes, sudden sharp pain with movement (round ligament pain)	Regular prenatal check-up with maternal vital signs, examination of the abdomen, sometimes pelvic examination, and measurement of fetal heart rate Sometimes ultrasonography of the pelvis
A <u>miscarriage</u>	Crampy pain in the pelvis Usually vaginal bleeding, sometimes with passage of tissue from the pregnancy	Fetal heart rate measurement A pregnancy blood test Ultrasonography of the pelvis
An <u>ectopic pregnancy</u> (an abnormally located pregnancy—not in its usual place in the uterus)	Abdominal or pelvic pain that • Is usually accompanied by vaginal bleeding • May be dull and come and go or may be severe, sudden, and constant If the ectopic pregnancy has ruptured, possibly fainting, light-	A pregnancy blood test Ultrasonography of the pelvis Sometimes laparoscopy (insertion of a viewing tube through an incision in the abdomen) or laparotomy (surgery involving an incision into the abdomen)



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